

Name: منى محمد صديق عبد الخالق
National ID: 28005272101041



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بان نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
--:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment

الوقت :

--:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تعريف الشخصية



منى
محمد صديق عبد الخالق
قرية الطرفاية
مركز البدرشين - الجيزة

١٩٨٠/٠٥/١٧

٢٨٠٠٥٢٧٢١٠١٠٤١

IV6639322

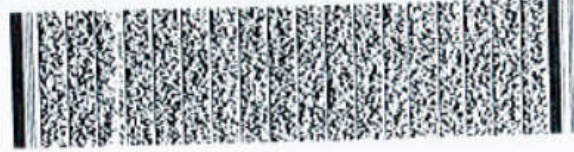


٢٨٠٠٥٢٧٢١٠٩٠٤١ ٢٠٢١/٠٣

ربة منزل



أنثى مسلمة
محمد محمود محمد طلبه
البطاقة سارية حتى ٢٠٢٨/٠٣/٢٣





Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:

1- Name : منى محمد صديق عبدالخالق

Sex:

3- Medical number:
Female

83072538

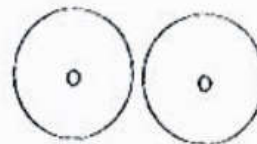
Old Med :

Old Med :

1508-2022

2-ID number: 28005272101041

Patient Age : 43 Y



Family history

Family History

Phone

Phone

01122530771

BMI

BMI

REASON OF MDT

REASON OF MDT
Reason

marital status

Marital Status

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

Co morbidity

Co Morbidity

Menopausal
status

Menopausal Status
Pre-menopausal

Initial STAGE

Initial STAGE

28/11/2023

Clinical data

Breast Imaging :

Mets Workup :

BX
IDC

ER
8/8

PR
4/8

Her 2 Neu

SISH

Ki67
35

BX COMMENT:
gll

ER COMMENT:

PR COMMENT:

Her 2 Neu COMMENT:
-ve

SISH COMMENT :

T

N

M

1 bone

Sonomammography

PATIENT HISTORY

pt received 4AC , 2 taxotere

bs: mets to bone \femara + zoladex fom 8/2022 till now

bs 11/2023:mild progression of previously detected osseous lesion of sternum , rt 6th rib , DV6

newly developed osseous deposit atb LV2,4

rt iliac bone

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

for faslodex - ibrance - zoladex

zometa

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

د. طارق حنين
د. محمد شهاب

RADIOLOGY

RADIOLOGY

28/11/2023

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition

Patient Name	: الإسماء / منى محمد عديق عبدالخالق	Request No.	: 13008472
Patient No.	: 28005272101041	Request Date	: 19/04/2022
Path No.	: 1060/22	Verification Date	: 08/05/2022
Sex	: Female	Financial Category	: فئة الدولة - أورام دار السلام
Age	: 41 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Lt breast mass

Nature of the specimen: Tru-cut biopsy, collected at 18/4/2022, 12:40am

Gross Examination:

- Procedure: Tru-cut biopsy. (Three greyish white tissue cores, 0.5 cm each, totally submitted.)
- Immunohistochemical technique: Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (SP1), PR clone (1E2), HER-2 CLONE (4B5) & Ki67 (30-9)

Microscopic examination:

- Histological type: Invasive Ductal Carcinoma
- Grade (differentiation): nuclear pleomorphism + mitosis: GII (3+2+1)
- Biomarkers

Examination of ER stained slide show intense nuclear staining in 70% of the tumor cells.
Examination of PR stained slide show faint nuclear staining in 30% of the tumor cells.
Examination of HER2 stained slide show no membranous staining in tumor cells.
Examination of Ki67 stained slide show nuclear staining in 35% of tumor cells.

Appropriate tissue control were reviewed

Diagnosis:

- Lt breast mass, Tru-cut biopsy
- Invasive Ductal Carcinoma GII
- ER: Positive (score 8/8).
- PR: Positive (score 4/8).
- HER-2: Negative (score 0).

Verified by:
dr.menna atef

Printed By : Ali mostafa

Page 1 of 2

Printing Date : 09.05.2022 09:18:11

Patient Name	الأستاذة / منى محمد صديق عبدالقادر	Request No.	13008472
Patient No.	28005272101041	Request Date	19/04/2022
Path No.	1060/22	Verification Date	08/05/2022
Sex	Female	Financial Category	توزيع دار السلام - فئة الدولة
Age	41 Y	Department	Out Patient
Referring Doc.		Registered by	marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Ki67: Positive in 35% of tumor cells.

Prof. Lubna Omar
Dr. Menna Atef

Verified by
dr.menna atef

Printing Date : 09/05/2022 09:18:11

Page 2 of 2

Printed By : A/ mostafa

Name : Mona Mohammed Sidik Abdelkhalek

Date : 04-06-2026 06:55

Branch : Dokki

Medical No. : 385850

Ref.Physician Prof : Goustav Roussey Egypt

Tc-99m MDP Whole-Body Skeletal Scintigraphy

Clinical History

- A 46-year-old female patient with a known history of left breast carcinoma presented for follow-up evaluation. The patient has a history of undergoing chemotherapy and radiotherapy, with no history of surgical tumor resection, bone surgeries, or recent skeletal trauma. She currently complains of localized bone pain.

Technique

- Following the intravenous injection of Tc-99m MDP, anterior and posterior whole-body scans were acquired approximately 2 hours post-injection.

Findings

- Comparison with the previous bone scan dated 06/10/2025 reveals the following:
- The previously described active metastatic deposits at the sternum, right 6th rib, DV6, LV5, right sacroiliac region, left acetabulum as well as proximal shaft of the left humerus are still noted, yet with time interval mild regression in tracer uptake intensity and a reduction in spatial resolution/extent, suggesting a favorable therapeutic response.
- No newly developed focal areas of abnormal tracer handling (hot or cold spots) are detected in the rest of the axial or appendicular skeleton.
- Normal physiological tracer distribution is noted within the kidneys and urinary bladder.
- Mild degenerative changes persist at the large joints and the remaining spine.

Impression

- Metastatic osseous lesions showing signs of mild regression, evidenced by a decrease in tracer uptake intensity and spatial resolution/extent compared to the prior study dated 06/10/2025, suggesting a positive response to ongoing treatment. No newly developed active deposits are identified.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine



SCAN 3 LABS

مركز النيل

Name : Mona Mohammed Sidik Abdelkhalck

Date : 06-10-2025 11:41

Branch : Dokki

Medical No. : 219127

Ref.Physician Prof : Gostaf Roseh Hospital

Tc^{99m} MDPSKELITAL SCINTIGRAPHY

History

- A 45-year-old patient, with history of breast cancer and referred for follow up.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings

Considering the prior study dated 09/11/2023:

- Still seen active lesions at sternum, right 6th rib, DV6, right sacro-iliac region and left acetabulum.
- Newly developed active osseous lesion at LV5 and proximal shaft of left humerus.
- No other abnormal active or cold focal bony lesions detected.
- Degenerative changes are noted at the spine and big joints.

Impression:

- Metastatic osseous lesions with progressive course evidenced by newly developed deposits as described.

Best regards,

Dr. Alaa Nagi, MD
Nuclear Medicine



Name : Mona Mohammed Sidik Abdelkhalek

Date : 06-10-2025 11:41

Branch : Dokki

Medical No. : 219127

Ref.Physician Prof : Gostaf Rosch Hospital

Tc^{99m} MDPSKELETAL SCINTIGRAPHY

History

- A 45-year-old patient, with history of breast cancer, and referred for follow up.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings

Considering the prior study dated 09/11/2023:

- Still seen active lesions at sternum, right 6th rib, DV6, right sacro-iliac region and left acetabulum.
- Newly developed active osseous lesion at L.V5 and proximal shaft of left humerus.
- No other abnormal active or cold focal bony lesions detected.
- Degenerative changes are noted at the spine and big joints.

Impression:

- Metastatic osseous lesions with progressive course evidenced by newly developed deposits as described.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine



Patient Name: Mona Mohamed Seddik Abdel Khalek	Date : 26/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 45 Year(s)
Medical ID:	Pat. ID: 28420

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

**** as compared to prior study dated 17/04/2024**

I Chest:

- **Rather stationary course of** left breast LOQ small soft tissue lesion is seen measuring about 2 x 1.2 cm in axial dimensions. Still noted skin thickening of the left breast.
- **Stationary course regarding size and number of :**
 - Prominent ipsilateral left axillary lymph nodes (level I & II), the largest at level I measures about 11 mm in short axis.
 - Bilateral upper lobes and middle lobe small pulmonary nodules (some of which show ground glass attenuation), the largest still noted at right upper lobe measures about 6 mm.
 - Bilateral predominantly lower lobes alternating areas of parenchymal veiling and air lucency.
 - Small mediastinal lymph nodes, largest at pre-carinal group about 10 mm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.

II Abdomen & pelvis:

Suboptimal study with incomplete pelvic cuts.

- The liver is **enlarged** with homogenous low parenchymal attenuation. No focal lesions (on non-contrast basis) or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- **Still noted** distended gall bladder showing multiple stones. Average mural thickness with no pericholecystic collection.
- **Mildly enlarged** spleen showing normal parenchymal with no focal lesions seen.
- Normal appearance of the pancreas, kidneys (apart from tiny left renal stone/gravel) and both suprarenals.
- Still noted prominent subcentimeteric para-aortic lymph nodes.
- **Still noted** left adnexal calcific lesions are seen.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic collections.
- **Still noted** distal descending colon circumferential mural thickening reaching 9 mm with notable surrounding fat smudging and stranding.

multiple osseous lesions are seen involving head of humerus, multiple dorsolumbar vertebra and pelvic bones vertebral bodies denoting good therapeutic response.

Opinion:

- ***Stationary course since the last study.***

Signature Reeham.Khader
29/10/2025 12:35 AM

MUHAMMID, MONA Patient ID: 122006358 DOB: 27-May-1980 Date: 09-Nov-2023
Patient Lastname: Muhammed
Patient Firstname: Mona
HIS Patient ID: 122006358
RIS Patient ID: 296916
Date of Birth: 27/05/1980

Procedure: 7C030, Bone Scan
Medical Question:
Clinical Comment:

===== REPORT TEXT =====

99mTc MDP Bone Scan Report (9/11/2023)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP Revealed:

Compared to previous bone scan done outside NCI dated 18/4/2023, today's study reveals:
" Mild progressive course of previously detected osseous lesions at the sternum, right 6th rib posteriorly, DV6 right sacro-iliac region and left acetabulum.
" New developed osseous deposits at LV2, 4 and right iliac bone.
" Both kidneys are visualized.

Conclusion:

Follow up bone scan reveals:

" Mild progressive course of previous detected bone lesions and new developed deposits as described.

Much Oblige
Rawan Sherra
Omnia Talaat, MD

===== REPORT TEXT ENDE =====



Patient Name: Mona Mohamed Seddik Abdel Khalek	Date : 02/12/2024
Exam : C.T ABDOMEN & PELVIS	Age : 44 Year(s)
Medical ID: 1508/2022	Pat. ID: 28420

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

***** as compared to prior study dated 04/07/2024***

I Chest:

- **Rather stationary course of** left breast LOQ small soft tissue lesion is seen measuring about 2 x 1.2 cm in axial dimensions. Still noted skin thickening of the left breast.
- **Stationary course regarding size and number of :**
 - Prominent ipsilateral left axillary lymph nodes (level I & II), the largest at level I measures about 11 mm in short axis.
 - Bilateral upper lobes and middle lobe small pulmonary nodules (some of which show ground glass attenuation), the largest still noted at right upper lobe measures about 6 mm.
- **Still noted**
 - Bilateral predominantly lower lobes alternating areas of parenchymal veiling and air lucency.
 - Small mediastinal lymph nodes, largest at pre-carinal group about 10 mm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.

II Abdomen & pelvis:

- The liver is **enlarged** with homogenous low parenchymal attenuation. No focal lesions (on non-contrast basis) or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.



Patient Name: Mona Mohamed Seddik Abdel Khalek	Date : 02/12/2024
Exam : C.T ABDOMEN & PELVIS	Age : 44 Year(s)
Medical ID: 1508/2022	Pat. ID: 28420

- **Still noted** distended gall bladder showing multiple stones. Average mural thickness with no pericholecystic collection.
- **Mildly enlarged** spleen showing normal parenchymal with no focal lesions seen.
- Normal appearance of the pancreas, kidneys (apart from tiny left renal stone/gravel) and both suprarenals.
- **Still noted** left adnexal calcific lesions are seen.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic collections.
- **Stationary course of** sclerotic osseous lesions are seen involving head of humerus , multiple dorsolumbar vertebra and pelvic bones vertebral bodies denoting good therapeutic response.

Opinion:

- ***Rather stationary course since the last study.***

Dr. Basma Mahrous

Prof. Menna Talla Hanafy, M.D.

Patient's Name:

Mona Mohammed Sidik Abdelkhalek

Date:

2026-09-07

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

**MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE NECK,
CHEST, ABDOMEN AND PELVIS AFTER ORAL AND I.V. CONTRAST
INJECTION REVEALED:**

**COMPARE TO PREVIOUS STUDY DATED 19/4/2026, THE CURRENT
STUDY REVELED**

I- NECK:

- Normal appearance of the naso-, oro- and hypopharynx with preserved parapharyngeal fat planes.
- Intact laryngeal skeleton with no glottic, supra or infra glottic masses.
- Normal course and caliber of the carotid and jugular vessels.
- Normal CT appearance of the submandibular and parotid salivary glands showing homogenous enhancement. No focal solid or cystic lesions.
- Few subscentemetric bilateral upper deep cervical and submandibular LNs are noted; likely non-specific.
- No pathologically enlarged cervical lymph nodes.
- Bulky thyroid gland for U/S correlation

II- CHEST:

- Mosaic attenuation of both lung fields with areas of air trapping
- Right few pulmonary nodules, largest seen at right middle lobe measuring about 4mm
- No consolidation or cavitation.
- No pathologically enlarged hilar or mediastinal lymph nodes.
- No plural or pericardial collection.
- CT signs of cardiomegaly

III- ABDOMEN AND PELVIS:

- enlarged liver showing homogenous low parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Spleen is enlarged in size
- Multiple gall bladder stones.

Patient's Name: **Mona Mohammed Sidik Abdelkhalek**

Date: **2026-09-07**

Referred by: **Prof.Dr. no doctor**

Hospital: **Goustave Roussey International Egypt**

- Right renal stone
- Small sliding hiatus hernia.
- Normal C.T appearance of the pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Left adnexal calcifications
- No abdominal or pelvic lymphadenopathy.
- No ascites.

IV- Skeletal survey:

- Still seen scattered vertebral sclerotic bony lesions

Opinion:

- ❖ Overall stationary course
- ❖ Cardiomegaly
- ❖ Fatty hepatomegaly.
- ❖ Calcualr gall bladder.
- ❖ Splenomegaly
- ❖ Right renal stone.
- ❖ Left adnexal calcifications

Dr.Nourhan Ewis

Name : Mona Mohammed Sidik Abdelkhalek
Branch : Dokki
Ref. Physician Prof : Goustav Roussey Egypt

Date : 04-06-2026 06:55
Medical No. : 385850

Tc-99m MDP Whole-Body Skeletal Scintigraphy

Clinical History

- A 46-year-old female patient with a known history of left breast carcinoma presented for follow-up evaluation. The patient has a history of undergoing chemotherapy and radiotherapy, with no history of surgical tumor resection, bone surgeries, or recent skeletal trauma. She currently complains of localized bone pain.

Technique

- Following the intravenous injection of Tc-99m MDP, anterior and posterior whole-body scans were acquired approximately 2 hours post-injection.

Findings

- Comparison with the previous bone scan dated 06/10/2025 reveals the following:
- The previously described active metastatic deposits at the sternum, right 6th rib, DV6, LV5, right sacroiliac region, left acetabulum as well as proximal shaft of the left humerus are still noted, yet with time interval mild regression in tracer uptake intensity and a reduction in spatial resolution/extent, suggesting a favorable therapeutic response.
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Impression

- Metastatic osseous lesions showing signs of mild regression, evidenced by a decrease in tracer uptake intensity and spatial resolution/extent compared to the prior study dated 06/10/2025, suggesting a positive response to ongoing treatment. No newly developed active deposits are identified.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine